Supplement Table 5: Findings of studies examining the association between occupational sitting and cardiovascular disease morbidity (CHD and stroke)

Nr.	Study	Sample	Major occupational groups	Length of follow-up/ duration	Assessment of occupational sitting (as analysed)	Outcomes[2]	Findings[3]	Study quality[4]	Sex-stratified analysis
	First author, year, country, cohort	n (%female) age at baseline	General population or specification		Categories of measurement and standardized instrument (if applicable)	(Data basis)	Statistical results and reference categories (e.g. risk estimates with 95 % confidence interval)	(++) (+) (0)	Yes/no
1.	Ferrario 2018 Italy MONICA, PAMELA, and SEMM-Study	n=3574 (0 % F); 25–64 y; mean age: 40.8 y	salaried employees (executive, white and bluecollar workers) and general population (incl. working)	14 y median	Baecke Physical Activity Questionnaire (BPAQ): self-reported OPA categorized in tertiles into low, moderate or high OPA after occupation type	Cardio-vascular diseases (CHD + stroke) (registries)	Reference intermediate OPA low OPA: HR=1.54 (1.04–2.23) **Compared to intermediate OPA, low OPA showed a higher CVD risk.**	++	Men only
2.	Hall 2017 USA Sister Study	n=31270 (100 % F); 35–74 y; no event mean age: 53.0 y with event mean age: 57.7 y	working population	(2005–2015) average 5.7 y (range 2.3–10.8)	Self-reported OPA at 4 intensity levels: mostly sitting; sitting and standing; mostly standing; high intensity work	TIA and stroke (self-reported doctor's diagnoses)	Reference mostly sitting in current job sitting/standing: HR=1.18 (0.93–1.49)[5] mostly standing: HR=1.04 (0.79–1.36)[5] High intensity: HR=1.12 (0.80–1.56)[5] **Compared with mostly sitting, no associations with sitting/standing or mostly standing OPA; but high intensity work increased TIA and stroke by 44 %.**	+	Women only

Legend:
n (% F): number of participants (percent female)
OPA: occupational physical activity
1 In alphabetical order and after methodological assessment rating (high to low quality)
2 Coronary heart disease (CHD), transient ischaemic attack (TIA)
3 In model adjusted for age, sex, leisure time physical activity (LPTA), health behaviour (alcohol, smoking, diet), and socioeconomic status if available, unless reported otherwise
4 Methodological Assessment incl. risk of bias: assessment after the SIGN-checklists resulting in overall ratings of 'high quality' (++), 'acceptable quality' (+) and 'unacceptable/low quality' (0), synonymic with 'high quality' (++) / low risk of bias, 'acceptable quality' (+) / accepable risk of bias, and 'unacceptable/low quality' (0) / unacceptable risk of bias.
5 Not adjusted for socioeconomic status
6 Adjustment for sex not reported